



Surgical Pathology Final Report

Temporary Copy

Case

Patient

Collected

ES

Ordered by

Location

Diagnosis

A. LEFT BREAST AND AXILLARY LYMPH NODES, MASTECTOMY AND LYMPHADENECTOMY:

- INVASIVE DUCTAL CARCINOMA, GRADE 2, 9.0 CM IN GREATEST DIMENSION.
- METASTATIC CARCINOMA IN TWO OF EIGHTEEN LYMPH NODES.
- MARGINS OF RESECTION ARE NOT INVOLVED.
- SEE SYNOPTIC REPORT AND NOTE.

NOTE: biomarkers studies have been ordered and will be reported in an addendum.

B. SKIN OF LEFT BREAST, ADDITIONAL INFERIOR, EXCISION:

- SEGMENT OF UNREMARKABLE SKIN.

(Electronic signature)

Verified:

ICD-O-3
Carcinoma, infiltrating duct NOS
Site @ Breast NOS
C50.9
JW 2/24/14

Synoptic Report

SPECIMEN:

Total breast (including nipple and skin)

PROCEDURE:

Total mastectomy (including nipple and skin)

LYMPH NODE SAMPLING:

Axillary dissection (partial or complete dissection)

SPECIMEN INTEGRITY:

Single intact specimen (margins can be evaluated)

SPECIMEN SIZE:

Greatest dimension: 27 cm

Additional dimensions: 19 x 11 cm

SPECIMEN LATERALITY:

Left

TUMOR SITE: INVASIVE CARCINOMA:

Upper inner quadrant

Lower inner quadrant

Central

TUMOR SIZE: SIZE OF LARGEST INVASIVE CARCINOMA:

Greatest dimension of largest focus of invasion over 0.1 cm: 9.0 cm

TUMOR FOCALITY:

Single focus of invasive carcinoma

MACROSCOPIC AND MICROSCOPIC EXTENT OF TUMOR:

Skin: Invasive carcinoma directly invades into the dermis or epidermis without skin ulceration

Nipple: DCIS does not involve the nipple epidermis

DUCTAL CARCINOMA IN SITU (DCIS):

DCIS is present

Extensive intraductal component (EIC) negative (small foci, less than 5% of the entire tumor)

ARCHITECTURAL PATTERNS:

Cribriform

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ID: [REDACTED]

Location: [REDACTED]

NUCLEAR GRADE:

Grade II (intermediate)

NECROSIS:

Not identified

LOBULAR CARCINOMA IN SITU (LCIS):

Not identified

HISTOLOGIC TYPE OF INVASIVE CARCINOMA:

Invasive ductal carcinoma (no special type or not otherwise specified)

GLANDULAR (ACINAR)/TUBULAR DIFFERENTIATION:

Score 3: <10% of tumor area forming glandular/tubular structures

NUCLEAR PLEOMORPHISM:

Score 2: Cells larger than normal with open vesicular nuclei, visible nucleoli, and moderate variability in both size and shape

MITOTIC COUNT:

Score 1

OVERALL GRADE:

Grade 2: scores of 6 or 7

MARGINS:

Margins uninvolved by invasive carcinoma

Distance from closest margin: 15 mm (deep fascial margin)

Margins uninvolved by DCIS (if present)

Distance from closest margin: 15 mm (deep fascial margin)

LYMPH-VASCULAR INVASION:

Present

DERMAL LYMPH-VASCULAR INVASION:

Not identified

LYMPH NODES:

Total number of lymph nodes examined (sentinel and nonsentinel): 16

Number of lymph nodes with macrometastases (>0.2 cm): 2

Size of largest metastatic deposit: 3.5 cm

EXTRANODAL EXTENSION:

Not identified

PRIMARY TUMOR (INVASIVE CARCINOMA (pT):

pT3: Tumor >50 mm in greatest dimension

REGIONAL LYMPH NODES (pN):

pN1a: Metastases in 1 to 3 axillary lymph nodes, at least 1 metastasis greater than 2.0 mm

DISTANT METASTASIS (M):

Not applicable

MICROCALCIFICATIONS:

Present in non-neoplastic tissue

CLINICAL HISTORY:

Palpable mass

Specimen Source

A LT Breast and Axillary Nodes

B Additional Inferior Skin

Clinical Information

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ID: [REDACTED]
Location: [REDACTED]

Patient with large mass upper Central breast, core biopsy positive

PRE-OP DIAGNOSIS: Left breast cancer

POST-OP DIAGNOSIS: Same

TYPE OF PROCEDURE: Left mass and axillary node dissection

Gross Description

The specimen is labeled "LEFT BREAST AND AXILLARY NODES" and is received in formalin. It consists of mastectomy specimen with lymph nodes weighing 1330 grams and measuring 27 x 19 x 11 cm with brown skin ellipse measuring 24 x 13 cm, containing grossly unremarkable 1.8 cm in diameter nipple. The posterior margin is composed of smooth fascia which is inked black. Superior non-deep margin is inked with red ink and the inferior non-deep margin is inked with blue ink. The breast is sliced in sagittal planes revealing a large well defined pink-tan lobulated mass measuring 9 x 7 x 6 cm and is located central portion of the breast and extending upper inner and lower inner quadrant. This mass is 1.5 cm away from the deep fascial margin of resection, 1.5 cm from the superior non-deep fascial margin and 2 cm from the inferior non-deep fascial margin of resection. The remaining portions reveal unremarkable yellow mammary fat with streaks of white-gray mammary parenchyma. The axillary fat pad measuring 10 x 10 x 5 cm. On sectioning multiple lymph nodes are identified. The largest lymph node consists of 2 matted lymph nodes measuring 4 x 3 x 3 cm and is pink tan and firm in consistency for 3.5 cm. Representative sections are submitted as follows:

A1 = nipple

A2-A5 = mass

A 6 = mass closest deep fascial margin of resection

A7 = representative sections upper outer quadrant

A8 = representative sections lower outer quadrant

A 9 = mass with closest skin surface

A 10 = six lymph nodes

A 11 = bisection of a single lymph node

A 12 = bisection of a single lymph node

A 13 = bisection of a single lymph node

A 14 = bisection of a single lymph node

A 15 = bisection of a single lymph node

A 16 = bisection of a single lymph node

A 17 = bisection of a single lymph node

A 18 = bisection of a single lymph node

A 19-A 20 = representative sections of 2 matted lymph nodes

Specimen is in formalin more than 6 hours and less than 48 hours

Time specimen was removed from the patient:

Time specimen was placed in formalin :

Ischemic time: 45 minutes

Dictated by:

Special Stains / Slides

23 H&E

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Ordered by: [REDACTED]

Patient: [REDACTED]

ID: [REDACTED]

Location: [REDACTED]

Tissue Code

Addendum Report Temporary Copy

Case: [REDACTED]
Collected: [REDACTED]
Ordered by: [REDACTED]

Patient: [REDACTED]
ID: [REDACTED]
Location: [REDACTED]

Addendum Report

IMMUNOHISTOCHEMICAL EVALUATION OF ESTROGEN RECEPTORS, PROGESTERONE RECEPTORS, AND HER-2NEU IN INVASIVE MAMMARY CARCINOMA .

ESTROGEN RECEPTORS: 99 %, positive, moderate staining intensity.
PROGESTERONE RECEPTORS: <1 %, negative, moderate staining intensity.

HER-2NEU: SCORE 1+, negative.

Immunohistochemical studies were performed on formalin fixed paraffin embedded tissue (Block A4) using the following monoclonal antibodies: Estrogen receptor (Clone SP1), Progesterone receptor (Clone 1E2) and Her-2neu (Clone 4B5); control sections for HER-2Neu are provided within a kit (score 0 MCF-7, score 1+ T-47D, score 2+ MDA-MB-453, score 3+ BT-474). Detection system used: polymer. Primary antibodies, reagents and control sections for HER-2neu are all provided by

All controls show appropriate reactivity.

Reactivity of Estrogen and Progesterone receptors is determined based on the percentage of positively stained nuclei of tumor cells. Reference values (CAP accreditation program checklist 2010 and guidelines on webpage):

Positive: nuclear staining in 1% or greater than 1% of invasive carcinoma cells

Negative : nuclear staining in less than 1% of invasive carcinoma cells

Staining intensity: is reported as weak, moderate or strong.

HER-2neu reactivity is reported applying the CAP scoring guidelines (CAP accreditation program checklist 2010 and guidelines on webpage):

Score 0 = Negative: No immunoreactivity, or faint weak immunoreactivity in <10% of tumor cells but only a portion of the membrane is positive..

Score 1 = Negative: Faint weak immunoreactivity in 10% or >10% of tumor cells but only a portion of the membrane is positive.

Score 2+ = Equivocal: Weak to moderate complete membrane immunoreactivity in >10% of tumor cells or circumferential intense membrane staining in <30% of cells.

Score 3+ = Positive: More than 30% of the tumor cells must show circumferential intense and uniform membrane staining. A homogeneous (chicken wire) pattern should be present.

Equivocal results for HER-2neu (Score 2+) will be subsequently followed by a reflex dual-color ISH testing.

The performance characteristics of these antibodies were determined by the

They have not been cleared or approved by the U.S. Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA-88) as qualified to perform high-complexity clinical laboratory testing.

(Electronic signature)

Verified:

Criteria	100 12/11/13	Yes	No
Diagnosis Discrepancy			/
Primary Tumor Site Discrepancy			/
HIPAA Discrepancy			/
Prior Malignancy History			/
Dual/Synchronous Primary Noted			/
Case is (circle):	QUALIFIED / DISQUALIFIED		
Reviewer Initials	DEE	Date Reviewed:	12/11/2013